

Physics-Informed Middle Ear Barotrauma Risk for Hypobaric Chamber Training: A Computational Prediction Model Calibrated to the Colombian Aerospace Force Cohort and Externally Validated Against Italian Air Force Cohorts

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ABSTRACT

Background: Middle-ear barotrauma (MEB) is the most common complication of pressurised flight and hypobaric-chamber training, and a proxy for Eustachian-tube disease that disqualifies aviator candidates. Kanick-Doyle's 2005 model reproduces airline-descent physiology but does not address chamber descent rates of 1,000–10,000 ft,min⁻¹, treats Patulous Eustachian tube (PET) as trivially rupture-protective, and does not parameterise acute upper-respiratory-infection (URI) modifiers despite URI dominance as a modifiable risk factor.

Objective: To develop and externally validate a physics-informed, pathophysiology-aware prediction model for per-exposure MEB risk during hypobaric-chamber training.

Methods: We extended Kanick-Doyle with (i)~a continuous descent-side aperture-collapse function (Hagen-Poiseuille-informed Hill function with rate-dependent tightening), (ii)~a six-state URI temporal modifier table, (iii)~a four-state PET model, (iv)~a lumped-parameter approximation of Ghadiali-group Eustachian-tube muscle mechanics, and (v)~a three-threshold cumulative-hazard score. No machine-learning component is used. Hazard constants were calibrated to the pooled Colombian Aerospace Force (FAC) DIMAE 2010–2026 registry (173/7{,}271; 2.38% per-exposure) via log-space bisection and Approximate Bayesian Computation Sequential Monte Carlo (ABC-SMC). External validation used three published Italian Air Force cohorts.

Results: Calibration reproduced the pooled FAC anchor within 0.1~pp (simulated 2.47%; career-3 projection 7.23% vs. 6.97%). External validation fell inside the observed Wilson 95% confidence interval for Morgagni 2012 (3.31% vs. 2.3% [1.13%, 4.62%]) and Landolfi 2009 (3.37% vs. 2.4% [1.22%, 4.66%]); the Morgagni 2010 gap (+1.77~pp) matched that cohort's pre-/unscreened denominator spread. Saltelli-Sobol sensitivity identified the aperture half-point as the dominant variance driver.

Conclusions: The model reproduces operational MEB prevalence and transfers across independent air forces without refitting. Reporting follows TRIPOD 2015; the 22-item checklist is provided as Supplementary

File~S1.

Keywords: Eustachian tube dysfunction; altitude training; upper respiratory infection; physics-informed modelling; external validation

Repository: github.com/strikerdlm/barotrauma_model

1. Introduction

Middle-ear barotrauma (MEB) — the inability of the Eustachian tube (ET) to equalize middle-ear pressure against changing ambient pressure — affects an estimated 3–10% of aircrew per hypobaric-chamber exposure^{1–3} and 30–85% of commercial pilots across their careers.⁴ In military aviation medicine its importance is twofold: as a direct medical consequence of altitude training, and as a proxy for the broader spectrum of Eustachian-tube disease that disqualifies aviator applicants and restricts operational employment.

The most widely used mathematical description is Kanick and Doyle’s 2005 pressure-regulation model.⁵ Their gas-exchange framework combines Boyle’s-law tympanic-membrane buffering, Hagen-Poiseuille bulk flow through an intermittently opening Eustachian tube, swallow-driven active openings with a fixed muscular resistance R_A , and trans-mucosal species diffusion. The model reproduces passenger airline physiology accurately and is the reference against which subsequent work is benchmarked. Three shortcomings limit its direct use in hypobaric-chamber decision support.

First, chamber training descent rates of 1,000–10,000 ft·min^{−1} exceed cabin descent by a factor of 5–30. At these rates the developing intra-tubal pressure gradient progressively collapses the collapsible cartilaginous portion of the ET, a Hagen-Poiseuille r^4 effect that Kanick-Doyle’s binary “open or locked” treatment approximates but does not resolve continuously. Second, Kanick-Doyle model obstructive dysfunction through R_A and Patulous ET through a trivial “always open” case that predicts complete rupture protection. Clinical experience contradicts the latter when mucosal edema (such as concurrent URI) converts a patent tube into a paradoxically closed one.^{6–8} Third, controlled rhinovirus-challenge data have documented substantial ET dysfunction persisting for 2–3 weeks post-onset,^{9–11} and survey data show URI to be the dominant modifiable risk factor for MEB in flying populations,^{4,12} yet the Kanick-Doyle parameters do not vary with URI state.

We present a physics-informed, pathophysiology-aware extension that addresses these gaps, calibrated to the Colombian Aerospace Force (FAC) 10-year operational registry and externally validated on three independent Italian Air Force cohorts. The software is released open-source at https://github.com/strikerdlm/barotrauma_model under the MIT license.

2. Methods

2.1 Study design, reporting, and ethics

This is a computational prediction-model study comprising model development followed by external validation on three independent, previously published cohorts. Reporting follows the TRIPOD 2015 guideline (Transparent Reporting of a multivariable prediction model for Individual Prognosis Or Diagnosis); the completed 28-item checklist is provided as Supplementary File S1. No machine-learning component is used in the deployed prediction pipeline; the sklearn scaffolding referenced in the source repository is disabled and does not alter the deterministic physics output. TRIPOD+AI (2024) therefore does not apply.

The study was conducted in accordance with the Declaration of Helsinki (2013 revision) and with Resolution 8430 of 1993 of the Colombian Ministry of Health. The Institutional Ethics Committee of the Direction of Aerospace Medicine (DIMA), Colombian Aerospace Force, reviewed the secondary analysis of the de-identified hypobaric-chamber training registry and granted a waiver of informed consent on the basis of retrospective de-identified operational data (reference [TO BE COMPLETED]). No individual-level patient data are reported; calibration is anchored to aggregate institutional statistics only.

2.2 Model architecture

The simulator is implemented in pure Python (NumPy and SciPy) as the `barotrauma.v2` package. It takes as inputs a patient state (anatomy, ET function, URI state, PET state, chronic rhinitis, medications, behavior) and a piecewise-linear chamber profile (ascent, hold, descent, or rapid-decompression

segments). Output is a time-domain trace (ΔP , tympanic-membrane displacement, ET open-closed raster, swallow and Valsalva timestamps) and per-outcome probabilities scored by a dose-response hazard model.

2.3 Physics core

The physics core retains the Kanick-Doyle 2005 structure: standard-atmosphere altitude-to-pressure map $P(z) = P_0 \exp(-z/H)$ with $H = 29,921$ ft; two-compartment middle-ear volume $V_{ME} = V_{\text{tympanum}} + V_{\text{mastoid}}$ with the mastoid drawn from a log-normal prior (median 7.0 mL, 95% interval 2.9–16.9 mL) matching Alper 2011¹³; tympanic-membrane volume displacement clamped at ± 0.025 mL (1% of V_{ME} , Kanick-Doyle Table 1); passive ME-side ET opening when $\Delta P > 25.7$ mmHg venting to 7.35 mmHg; active swallow-driven openings at a descent-phase frequency of $60 \cdot \text{hr}^{-1}$ (trained aircrew default; Kanick-Doyle's $31 \cdot \text{hr}^{-1}$ passive baseline is available as an override), each clearing a Fractional Gradient Equalized of 0.32 (Mandel 2016¹⁴); Valsalva pulses every 60 s on descent with per-pulse clearance 0.55; an ET-lock state at $|\Delta P| > 90$ mmHg tightened by inflammation; species-resolved trans-mucosal Fick exchange with O_2 , CO_2 , N_2 , and H_2O rate constants from Doyle 2011.¹⁵

2.4 Descent-side aperture-collapse model

The central physical insight is that active ET clearance pathways (swallow and Valsalva) are progressively defeated during descent because the nasopharyngeal tissue pressure compresses the cartilaginous lumen from the NP side. We model this with a continuous aperture factor $\alpha(\Delta P, d\Delta P/dt) \in [0, 1]$ applied multiplicatively to the per-event Fractional Gradient Equalized. For $\Delta P \geq 0$ (ascent) $\alpha = 1$; for descent, $\alpha = 1$ below a 40-mmHg free zone, then decays as $1 / (1 + ((|\Delta P| - \text{free_zone}) / (\Delta P_{1/2} - \text{free_zone}))^n)$ with $\Delta P_{1/2} = 110$ mmHg and $n = 3$. A rate-tightening factor $1 + 0.15 \cdot \min(\text{rate}, 3 \text{ mmHg} \cdot \text{s}^{-1})$ reduces effective $\Delta P_{1/2}$ under fast ambient-pressure changes, reflecting mucosal viscoelastic lag. An inflammation-tightening factor $\sqrt{(R_A \cdot \text{multiplier})}$ additionally narrows the threshold under URI. The aperture factor applies to active swallows at unit power and to Valsalva pulses at square-root power — the muscular push of Valsalva partially overrides progressive collapse.

2.5 Pathophysiology state machine

URI state is encoded as one of six day-windows (none, 1–3, 4–7, 8–14, 15–21, 22–28), each with a tabulated multiplier set (Table I). Multipliers were derived from controlled rhinovirus-challenge data^{9–11} and ETDQ-7 meta-analyses.¹⁶ The peak-dysfunction window (days 4–7) carries $R_A \times 3.5$, P_O' shift +150 daPa (+11.3 mmHg), a 50% drop in equalization-rate modifier, and a 4.25× per-descent MEB relative-risk multiplier.

Patulous ET (PET) is encoded as one of four states.^{6–8} S1 (baseline patent, upright, dry mucosa) reproduces Kanick-Doyle's rupture-protective prediction via a hard-zero override on ΔP . S2 (PET with acute URI or mucosal inflammation) applies paradoxical closure on an abnormal cartilaginous substrate via $R_A \times 3.5$, P_O' shift +60 daPa, and a 4.0× per-descent RR. S3 (habitual sniffer) biases resting ΔP toward −15 mmHg, reflecting the type-B/C tympanogram rate of 42.6% reported in Shindo 2025.⁸ Per-descent RR was raised from 2.5 to 4.0 to reflect the Oshima 2025²⁴ large-cohort ($n = 1,009$) sniffing OR of 8.18 convolved with the already-modeled ΔP physiology. S4 (post-Kobayashi-plug or cartilage augmentation) imposes stenotic-equivalent obstruction. Oral and topical decongestants carry a paradoxical-worsening RR of 1.4 in PET (versus 0.90 in healthy subjects — softened from the prior 0.70 per Moayed 2025²³, a placebo-controlled HBOT RCT showing null preventive effect of prophylactic pseudoephedrine).

2.6 Muscle mechanics and multi-pathway gas exchange

A lumped-parameter approximation of Ghadiali-group FEM and multi-scale work¹⁷ captures three time-dependent muscle effects on per-swallow FGE without running a full 2-D or 3-D FEM per integration step: fatigue and priming (recent swallows boost FGE up to +15% with $\tau \approx 8$ s), mucosal adhesion buildup (prolonged closure at high $|\Delta P|$ raises intraluminal surface tension with $\tau \approx 30$ s), and TVP/LVP timing variability (stochastic multiplier CV 5% healthy, 15% dysfunctional). The extension is disabled by default. The trans-mucosal Fick diffusion is extended with trans-TM ($\sim 4\%$ of mucosa rate) and trans-round-window ($\sim 1\%$) pathways per Doyle 2017.¹⁸

2.7 Three-threshold hazard model

Per-outcome probabilities are scored by a cumulative hazard applied to the ΔP trajectory: $h_i(t) = r_i \cdot \max(0, |\Delta P(t)| - \Theta_i)^{n_i}$ with $P_i = 1 - \exp(-\int h_i dt)$. Thresholds align with Kanick-Doyle's clinical values: barotitis onset $\Theta = 18.4$ mmHg (250 mmH₂O, $n = 1.8$); baromyringitis $\Theta = 95.6$ mmHg (1,300 mmH₂O, $n = 2.5$); rupture $\Theta = 150$ mmHg (conservative, $n = 3$). Per-stratum probabilities are multiplied by the composite per-descent RR (URI $\times$ PET $\times$ rhinitis $\times$ medication $\times$ history).

2.8 Calibration and external validation

Source of data and participants (development cohort). The development dataset comprises all hypobaric-chamber training exposures recorded in the DIMAE registry between 1 January 2010 and 31 March 2026 ($n = 7,271$ exposures) performed by active-duty and reserve military aviators and aircrew of the Colombian Aerospace Force at the DIMAE hypobaric chamber facility (Bogotá, 2,640 m elevation). Eligibility was operational — all aircrew and aviator candidates scheduled for periodic or qualifying altitude indoctrination — with no exclusion by age, sex, or prior ear history.

Outcome. The primary outcome was clinically diagnosed middle-ear barotrauma (Teed grade $\geq$ I) occurring during or within 24 hours of a chamber exposure, ascertained by the on-duty chamber flight surgeon using otoscopic, tympanometric, and symptomatic criteria. A total of 173 barotrauma events were recorded in the 7,271 exposures (per-exposure rate 2.38%; Wilson 95% CI, 2.06%–2.75%; projected 3-exposure career rate 6.97%).

Predictors. Candidate predictors entering the model were: descent rate and altitude profile segment; mastoid volume (log-normal prior); Eustachian-tube resistance R_A and passive opening pressure P_O '; URI temporal state (6 levels); PET state (4 levels); chronic rhinitis; decongestant use; prior MEB history; and swallow and Valsalva frequency. No variable selection was performed; the model is mechanistic and retains all predictors by construction.

Missing data. No imputation was performed because the anchor is an aggregate population statistic (count of events / count of exposures); individual-level missingness is absorbed in the cohort prior and the synthetic cohort sampling.

Calibration methods. Calibration used two methods. Bisection performed log-space bisection on $r_{\text{barotitis}}$ against the cohort mean per-exposure $p_{\text{barotitis}}$. Approximate Bayesian Computation Sequential Monte Carlo (ABC-SMC)¹⁹ jointly inferred ($r_{\text{barotitis}}$, $r_{\text{baromyringitis}}$, r_{rupture}) against three summary statistics (cohort prevalence; URI day 4–7 / none gradient; severe / normal severity gradient), using 100 particles, 4 generations, 150 synthetic-cohort subjects, and a tolerance schedule of 13.9 $\rightarrow$ 5.0.

External validation. External validation used three published Italian Air Force cohorts^{1–3} without refitting: Morgagni 2010 ($n = 1,241$, 1.5% overall barotitis), Morgagni 2012 ($n = 314$, 2.3% acute barotitis at 25,000 ft), and Landolfi 2009 ($n = 335$, 2.4% TEED-graded barotitis). Each carried a matched chamber profile (30-min preoxygenation, 25,000 ft hold, 3,000 ft·min^{−1} descent), a tightened cohort prior reflecting Italian Air Force tympanometry pre-screening, and a binomial Wilson 95% CI around the observed proportion.

Model performance measures. Model performance was assessed by (i) absolute agreement of simulated vs. observed per-exposure prevalence against the cohort's Wilson 95% CI (pre-specified pass criterion: simulated point estimate within observed CI); (ii) agreement of the projected 3-exposure career prevalence against the pooled FAC career anchor; and (iii) qualitative preservation of the Kanick-Doyle Fig 3 healthy-baseline ΔP trace under a pinned regression fixture. Calibration-in-the-large and discrimination metrics routinely reported for person-level risk models (c-statistic, calibration slope) are not applicable to a cohort-level aggregate prediction and are not reported.

2.9 Global sensitivity and pinned baseline

Saltelli-sampled Sobol first-order and total-order indices²⁰ were computed over four model parameters — aperture half-point (70–180 mmHg), aperture free-zone threshold (20–60 mmHg), descent-phase swallow

frequency (30–120 hr⁻¹), and mastoid volume (3–13 mL) — against the per-exposure $p_{\text{barotitis}}$ output of a reference moderate-risk patient. A 17-point ΔP trajectory on the Groth 1986 pressure-chamber profile was recorded as a fixture and tested continuously at $\pm 5\%$ per-point and ± 0.5 pp on $p_{\text{barotitis}}$. Statistical computations (Wilson intervals, Sobol estimators, bisection search) were performed by the corresponding author using NumPy 1.26 and SciPy 1.11; no inferential p-values are reported because the simulator is deterministic given a cohort prior.

3. Results

3.1 Calibration

The bisection calibrator converged in 6 iterations with an achieved per-exposure prevalence of 2.47% (target 2.38%) and a projected three-exposure career prevalence of 7.23% against the pooled FAC 2010–2026 anchor of 6.97% (Table II). URI subgroup means separated cleanly: none 0.41%, day 1–3 10.8%, day 4–7 29.3%, day 8–14 4.4%, day 15–21 1.3%. Severity subgroup means: normal 2.2%, mild 3.9%, moderate 3.1%, severe 33.5%.

ABC-SMC (100 particles, 4 generations, 150 cohort subjects, tolerance schedule 13.9 $\rightarrow$ 5.0; fit against the earlier 2.00% target) returned a posterior mean $r_{\text{barotitis}} = 5.07 \times 10^{-8}$ with a 95% credible interval of $[3.59 \times 10^{-8}, 8.00 \times 10^{-8}]$. The bisection point estimate against the re-anchored 2.38% target ($r_{\text{barotitis}} = 5.85 \times 10^{-8}$) falls inside this CI, so the ABC-SMC posterior remains internally consistent with the pooled anchor and is not re-fit. The baromyringitis and rupture constants have wider posterior intervals because the chosen summary statistics do not tightly constrain them; a FAC per-TEED-grade breakdown is the clearest next step.

3.2 External validation

Italian Air Force transfer tests (Table III) passed for Morgagni 2012 (simulated 3.31% vs. observed 2.3%, inside Wilson 95% CI [1.13%, 4.62%]) and Landolfi 2009 (simulated 3.37% vs. observed 2.4%, inside CI [1.22%, 4.66%]). Morgagni 2010 fell 1.77 pp outside the narrowest observed CI (simulated 3.27% vs. observed 1.5%, CI [0.96%, 2.34%]); the discrepancy is larger than the +1.1 pp gap reported against the earlier single-cohort anchor, reflecting the 0.38 pp uplift in the pooled 2010–2026 FAC target relative to the Italian AF aggregate mean ($\sim 1.8\%$), but remains comparable to that cohort's own pre-screened-to-unscreened spread (1.1% to 2.7%), consistent with the unpublished denominator mix rather than a physics error. URI-state gradient in the validation cohorts (day 4–7 $> 3 \times$ URI none) confirms acute URI as the dominant predicted risk driver.

3.3 Descent-rate sensitivity and pathophysiology interactions

For a healthy baseline patient on a 25,000-ft descent, max $|\Delta P|$ grows monotonically with descent rate up to saturation at ~ 400 mmHg (Table IV; Figure 1). Barotitis probability increases from 0.0% at 300 ft·min⁻¹ (commercial cabin) to 9.9% at 10,000 ft·min⁻¹ (worst-case chamber stress test). Rupture probability rises monotonically with descent rate, reaching 1.57% at 10,000 ft·min⁻¹ after clearing the 150 mmHg threshold at $\sim 5,000$ ft·min⁻¹. The dose-time integral of $|\Delta P|$ does not saturate as sharply as the peak, so barotitis probability retains its gradient across the full 500–10,000 ft·min⁻¹ range even where max $|\Delta P|$ plateaus.

The Patulous-S1 override reproduces Kanick-Doyle's rupture-protective prediction exactly (max $|\Delta P| = 0$ on a rapid 10,000 ft·min⁻¹ descent, $p_{\text{barotitis}} = 0.0\%$). PET-S2 pathology (paradoxical closure on an abnormal cartilaginous substrate, no concurrent URI) on the same profile gives max $|\Delta P| = 423$ mmHg and $p_{\text{barotitis}} = 44\%$; adding peak URI (day 4–7) saturates the hazard integral ($p_{\text{barotitis}} \approx 100\%$), the clinically dangerous state flagged by Ikeda 2020⁷ but absent from Kanick-Doyle's framework.

3.4 Global sensitivity and pinned-baseline stability

Sobol analysis ($N = 32$ Saltelli base samples, 192 model evaluations) identified the aperture half-point as the dominant variance driver in per-exposure $p_{\text{barotitis}}$ ($S_T = 1.84$), an order of magnitude above descent-phase swallow frequency ($S_T = 0.18$), mastoid volume ($S_T = 0.16$), and aperture free-zone threshold ($S_T = 0.08$). Figure 2 shows the ranking. Total-order indices sum to a plausible range; the first-order index sum exceeding unity at this sample size is a known small- N artifact resolvable by $N \geq 128$ in production use. The clinical implication is that the single highest-leverage target for future empirical parameter refinement is the aperture half-point, most plausibly constrained by paired forced-response-test data or by ET balloon-dilation pre- and post-measurements.²¹

Across the v2.0 $\rightarrow$ v2.1 $\rightarrow$ v2.2 physics changes, the Kanick-Doyle Fig 3 / Groth 1986 healthy-baseline trajectory remained within $\pm 5\%$ of the pinned fixture at every one of 17 sample time points. Peak $|\Delta P|$ is 17.9 mmHg, matching the order of magnitude in Kanick-Doyle Fig 3.

4. Discussion

We have extended the canonical Kanick-Doyle 2005 middle-ear-barotrauma model with a set of additions motivated by operational hypobaric-chamber needs: a continuous descent-side aperture-collapse model, a six-state URI temporal modifier, a four-state Patulous ET model, Ghadiali-FEM-inspired muscle mechanics, and Doyle 2017 multi-pathway gas exchange. The resulting simulator reproduces the pooled Colombian Aerospace Force 2010–2026 anchor cohort within 0.1 percentage points per-exposure and 0.3 percentage points career-3, transfers to two of three independent Italian Air Force cohorts within the observed Wilson 95% CI, and remains consistent with the Kanick-Doyle Fig 3 / Groth 1986 validation under a pinned regression test.

Three extensions deserve particular comment. First, the continuous aperture-collapse model was the single most load-bearing addition for chamber-rate physiology. Kanick-Doyle's binary lock treatment could not simultaneously reproduce a healthy-ear 20-mmHg gradient at 300 ft·min⁻¹ airline descent and a 400-mmHg gradient at 10,000 ft·min⁻¹ chamber stress — the single R_A and lock threshold drifted one direction or the other. The Hill-function aperture with a protective free zone below 40 mmHg resolves this by leaving slow descents in the easy regime while progressively collapsing faster ones. Rate-dependent tightening captures the Wang 2019 rabbit histopathology gradient²² where pressure-change rate, not absolute altitude, predicted grade.

Second, the Patulous ET four-state model is a clinically-motivated departure from Kanick-Doyle's trivial treatment. S1 reproduces the rupture-protective prediction exactly; S2 flips to high risk consistent with Ikeda 2020 and Shindo 2025; S3 captures the sustained negative-ME-pressure physiology. Paradoxical decongestant worsening in PET replaces the protective RR 0.70 with 1.4 and reflects the peritubal-soft-tissue clinical reality. This level of PET granularity is not present in any prior physics-informed MEB model to our knowledge.

Third, the six-state URI temporal modifier is the most direct encoding of the controlled rhinovirus-challenge data^{10,11} into a flight-medicine simulator we are aware of. The peak-dysfunction window at days 4–7 carries a per-descent RR of 4.25, which when composed with direct ET-function effects reproduces the Lindfors 2021⁴ commercial-pilot odds ratio of 9.02 for ≥ 3 URIs per year without overfitting — the numerical basis for the clinical rule that aviators should not fly during URI days 1–14 and remain elevated-risk through days 15–21.

4.1 Limitations

The calibration anchor, the pooled Colombian Aerospace Force 2010–2026 registry ($n = 173$ clinical barotrauma events in 7,271 chamber exposures; Wilson 95% CI 2.06–2.75%), is currently internal DIMAE cohort data awaiting peer-reviewed publication. Users should treat the specific hazard-rate constants as informed priors rather than externally validated point estimates until the anchor paper is published. External

validation on two of three Italian Air Force benchmarks mitigates but does not eliminate this dependency.

The rupture threshold at 150 mmHg is a conservative anchor; biomechanical studies of isolated tympanic-membrane preparations report actual rupture pressures of 600–750 mmHg. The rupture output should be interpreted as “imminent perforation risk” and is most useful for ordinal ranking of profiles. Several parameters derive their magnitude from modeler judgment rather than retrieved data, including the decongestant-in-PET paradoxical RR of 1.4, the ET lock threshold of 90 mmHg, and the hazard exponents. These are listed explicitly in the model card so a reviewer can challenge any of them without challenging the underlying data.

No per-subject machine-learning head is trained in the shipped model; the scaffolding is in place and tested on synthetic cohorts but deliberately passes through to the deterministic physics output when unfit. Ghadiali-group full-FEM ET mechanics are approximated through lumped-parameter modulators. Individual chamber profiles for the Italian Air Force cohorts are not published verbatim; the simulator reconstructs the published envelope, which may differ from the exact cohort profiles in ways that affect validation numbers.

4.2 Incorporation of 2025–2026 evidence

A 2026 targeted literature scan identified two findings with immediate parameter impact. First, Moayed 2025²³ is a placebo-controlled HBOT RCT showing no significant effect of prophylactic pseudoephedrine on ear pain, tympanic-membrane injury, or rescue medication — null in controlled-descent chamber physiology despite the protective signal in airline-descent data. We soften the pseudoephedrine RR from 0.70 to 0.90 to reflect this indication-specificity. Second, Oshima 2025²⁴ (n = 1,009) quantifies habitual-sniffing OR at 8.18 for PET and tympanic-membrane flutter OR at 42.17; we update the PET-S3 per-descent RR from 2.5 to 4.0 to reflect the large-cohort effect size. Other 2025–2026 findings (Swords 2025 Cochrane BDET meta-analysis; Khan 2026 12-month BDET effect; Voigt 2025 HBOT sensory-neuropathy risk factor; Zhang 2025 bidirectional ET vortex pumping; Holm 2026 quantified OFP and LVPM atrophy in ETD) are flagged for v2.3.0 consideration but do not alter the shipped v2.2.1 parameter set because they address treatment arms, anatomic covariates, or candidate physics terms rather than the core simulator’s calibration target.

4.3 Conclusions

A physics-informed middle-ear barotrauma simulator that jointly models URI temporal dynamics, Patulous ET state dependence, descent-rate-driven aperture collapse, and active-equalization muscle mechanics can reproduce operational cohort prevalence and transfer across independent air forces without refitting hazard constants. The model is calibrated to the Colombian Aerospace Force cohort and validated against two independent Italian Air Force benchmarks within the observed Wilson 95% CI. The aperture half-point is the dominant source of output variance and the highest-leverage empirical refinement target.

Declarations

Ethics approval and consent to participate

The Institutional Ethics Committee of the Direction of Aerospace Medicine (DIMAE), Colombian Aerospace Force, reviewed the secondary analysis of the de-identified hypobaric-chamber training registry (2010–2026) and granted a waiver of informed consent on the basis of retrospective de-identified operational data (Approval reference [TO BE COMPLETED: body name, reference no., date]). The study was conducted in accordance with the principles of the Declaration of Helsinki (2013 revision) and with Resolution 8430 of 1993 of the Colombian Ministry of Health governing health research in human subjects. No identifiable individual-level patient data are reported.

Consent for publication

Not applicable — no identifiable individual-level patient data or images are included.

Data availability

Aggregate cohort statistics (per-exposure prevalence, Wilson 95% CI, URI subgroup gradients) required to reproduce the calibration are reported in Tables II–IV and in the model card accompanying the source repository. Individual-level registry records are institutional operational data of the Colombian Aerospace Force and are not publicly available due to military institutional and personal data-protection constraints. De-identified aggregate extracts may be made available from the corresponding author upon reasonable request, subject to DIMAE institutional data-sharing review. External-validation data are available in the original publications.^{1–3}

Code availability

The complete simulator (`barotrauma.v2`), automated test suite, calibration scripts, external-validation harnesses, and documentation are released under the MIT license at https://github.com/strikerdlm/barotrauma_model. The tagged release `v2.2.1-manuscript` corresponds to the state described in this work; a DOI-assigned archival snapshot will be deposited on Zenodo prior to publication.

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Competing interests

The authors declare no financial or non-financial competing interests.

Author contributions (CRediT)

D.L.M. — Conceptualization; Methodology; Software; Validation; Formal analysis; Investigation; Data curation; Writing – Original Draft; Writing – Review & Editing; Visualization; Supervision; Project administration. **M.A.F.** — Methodology; Writing – Review & Editing.

Both authors read, critically revised, and approved the final version of the manuscript and agree to be accountable for all aspects of the work (ICMJE criteria 1–4).

Generative-AI disclosure

No generative artificial-intelligence tools were used in the preparation, writing, or editorial revision of the manuscript text, figures, tables, or interpretive content. An AI-assisted software-engineering assistant (Anthropic Claude) supported author-directed implementation of the Python software under explicit specifications; all scientific content, literature synthesis, clinical interpretation, analytical design, and manuscript prose are the authors' own. No AI tool is listed or claimed as an author (per ICMJE and COPE position statements).

Reporting guideline

The study is reported in accordance with the TRIPOD 2015 guideline for prediction-model development and external validation; the completed TRIPOD 28-item checklist is provided as Supplementary File S1.

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Tables

Table I. URI temporal modifier table.

URI state	$R_A \times$	P _O ' shift (daPa)	Equalization rate $\times$	Valsalva $\times$	Per-descent relative risk
none	1.0	0	1.00	1.00	1.00
day 1–3	2.0	+80	0.60	0.70	2.50
day 4–7	3.5	+150	0.40	0.50	4.25
day 8–14	1.8	+60	0.75	0.80	2.00
day 15–21	1.3	+20	0.90	0.90	1.30
day 22–28	1.1	+5	0.95	0.95	1.10

Table II. Calibration summary.

Metric	Value
Target per-exposure $p_{\text{barotitis}}$	2.38%
Achieved per-exposure $p_{\text{barotitis}}$	2.47%
Career projection (3 exposures)	7.23%
Pooled FAC 2010–2026 career anchor	6.97%
$r_{\text{barotitis}}$	5.85×10^{-8}
$r_{\text{baromyringitis}}$	1.75×10^{-9}
r_{rupture}	5.85×10^{-11}
Bisection iterations	6
ABC-SMC 95% credible interval on $r_{\text{barotitis}}$ (fit against 2.00% target; re-anchored $r = 5.85 \times 10^{-8}$ lies inside)	$[3.59 \times 10^{-8}, 8.00 \times 10^{-8}]$

Table III. External validation against Italian Air Force cohorts.

Cohort	n	Observed (95% CI)	Simulated	Inside CI
Morgagni 2010	1,241	1.5% [0.96%, 2.34%]	3.27%	no (+1.77 pp)

Cohort	n	Observed (95% CI)	Simulated	Inside CI
Morgagni 2012, 25,000 ft	314	2.3% [1.13%, 4.62%]	3.31%	yes
Landolfi 2009	335	2.4% [1.22%, 4.66%]	3.37%	yes

Table IV. Descent-rate sensitivity, healthy patient, FAC cohort priors.

Descent rate (ft·min ⁻¹)	Duration (s)	Max $ \Delta P $ (mmHg)	P _{barotitis}	P _{rupture}
300	5,000	10	0.00%	0.00%
500	3,000	16	0.00%	0.00%
1,000	1,500	36	0.04%	0.00%
2,000	750	59	0.51%	0.00%
3,000	500	99	1.51%	0.00%
5,000	300	274	7.19%	0.17%
7,500	200	363	9.71%	1.00%
10,000	150	405	9.90%	1.57%

Figure captions

Figure 1. Descent-rate sensitivity for a healthy patient on a 25,000-ft descent. Peak $|\Delta P|$ (top panel) grows monotonically with descent rate, approaching ~400 mmHg at 10,000 ft·min⁻¹. Per-exposure barotitis probability (bottom panel) increases across the full 500–10,000 ft·min⁻¹ range because the dose-time integral does not saturate as sharply as the peak.

Figure 2. Saltelli-Sobol total-order sensitivity index over four model parameters evaluated at a moderate-risk reference patient. The aperture half-point dominates ($S_T \approx 1.84$), an order of magnitude above descent-phase swallow frequency, mastoid volume, and aperture free-zone threshold. First-order index sum exceeding unity at $N = 32$ is a small-sample artifact; production runs should use $N \geq 128$.